



## MIST- MEDICAL INSTITUTE FOR SCREENING TEST

### **MOCK TEST - 8 PAPER - 1 DATED ON 20th JUNE 2026**

#### **QUESTIONS**

- Q1. A 68-year-old man presents with a progressively enlarging right groin swelling that has developed gradually over several years. He reports chronic cough related to COPD, long-standing constipation, and urinary hesitancy requiring straining to void. The swelling becomes more prominent at the end of the day and enlarges significantly during coughing or prolonged standing. He occasionally experiences a dragging discomfort radiating toward the medial thigh but denies bowel obstruction symptoms.

On examination, the swelling is located superior and medial to the pubic tubercle. It is reducible when supine. After reduction, the examiner invaginates the scrotal skin toward the superficial inguinal ring and asks the patient to cough. The impulse is felt against the pad of the finger rather than the fingertip. Compression over the deep inguinal ring fails to prevent recurrence of the swelling. The mass rarely descends completely into the scrotum.

CT imaging demonstrates protrusion of peritoneal sac and preperitoneal fat through a weakened area bounded by:

the lateral border of rectus abdominis,  
inferior epigastric vessels,  
and the inguinal ligament.

Which of the following best explains the anatomical basis of this patient's condition?

- a) Herniation through the deep inguinal ring lateral to inferior epigastric vessels due to persistence of processus vaginalis
- b) Acquired protrusion through weakened transversalis fascia within Hesselbach triangle medial to inferior epigastric vessels
- c) Herniation below the inguinal ligament through the femoral canal medial to the femoral vein
- d) Protrusion through the semilunar line lateral to the rectus sheath beneath intact external oblique aponeurosis

- Q2. A 41-year-old man develops progressive ophthalmoplegia and facial sensory symptoms several days after a facial infection. Imaging reveals thrombosis involving a venous sinus adjacent to the sella turcica. Which of the following cranial nerves is most vulnerable because it travels centrally within the sinus beside the internal carotid artery rather than within the lateral wall?

- a) Oculomotor nerve — supplies most extraocular muscles and travels within the lateral wall of the cavernous sinus, producing ptosis and “down-and-out” eye when injured
- b) Trochlear nerve — supplies superior oblique muscle and courses within the lateral wall of the cavernous sinus, producing vertical diplopia when affected
- c) Ophthalmic division of trigeminal nerve — provides afferent limb of corneal reflex and travels in the lateral wall of the cavernous sinus
- d) Abducens nerve — innervates lateral rectus muscle and passes centrally through the cavernous sinus adjacent to the internal carotid artery, making it highly vulnerable in cavernous sinus thrombosis



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### **MOCK TEST - 8 PAPER - 1 DATED ON 20th JUNE 2026**

- Q3. A 52-year-old woman develops progressive voice changes following neck surgery. Flexible laryngoscopy demonstrates unilateral vocal cord paralysis. Which of the following nerves is most likely injured?
- a) External branch of superior laryngeal nerve — innervates cricothyroid muscle and injury causes inability to tense vocal cords with loss of high-pitched phonation
  - b) Recurrent laryngeal nerve — supplies all intrinsic laryngeal muscles except cricothyroid and is closely related to the inferior thyroid artery during thyroid surgery
  - c) Glossopharyngeal nerve — supplies stylopharyngeus and sensory innervation to posterior one-third of tongue, causing dysphagia and loss of gag reflex when injured
  - d) Hypoglossal nerve — supplies intrinsic and extrinsic tongue muscles causing tongue deviation toward side of lesion
- Q4. A 63-year-old man with poorly controlled hypertension suddenly develops difficulty speaking and weakness of the right upper and lower limbs. Neurological examination reveals deviation of the tongue toward the left on protrusion. There is spastic paralysis of the right side of the body with hyperreflexia and extensor plantar response. Vibration and proprioception are impaired on the right side, while pain and temperature sensation remain intact. Which vascular territory is most likely affected?
- a) Posterior inferior cerebellar artery — involving nucleus ambiguus, spinal trigeminal nucleus, and spinothalamic tract causing lateral medullary syndrome
  - b) Anterior spinal artery — involving pyramids, medial lemniscus, and hypoglossal nerve fibers causing medial medullary syndrome
  - c) Anterior inferior cerebellar artery — involving facial nucleus and vestibulocochlear structures causing lateral pontine syndrome
  - d) Posterior cerebral artery — involving cerebral peduncle and oculomotor fibers causing Weber syndrome
- Q5. A 29-year-old man sustains a displaced fracture of the humeral shaft after falling from a ladder. Several days later, he notices weakness while extending his wrist and fingers. On examination, he is unable to extend the wrist against resistance, and the hand assumes a flexed posture at rest. Sensation is diminished over a small dorsal area between the thumb and index finger. Triceps strength is largely preserved. Which of the following nerves is most likely injured?
- a) Axillary nerve — courses through quadrangular space with posterior circumflex humeral artery and supplies deltoid and teres minor
  - b) Musculocutaneous nerve — pierces coracobrachialis and supplies anterior compartment of arm with sensation over lateral forearm
  - c) Radial nerve — traverses the spiral groove of the humerus with profunda brachii artery and supplies extensor compartments of arm and forearm
  - d) Ulnar nerve — passes posterior to medial epicondyle and supplies most intrinsic hand muscles



## **MIST- MEDICAL INSTITUTE FOR SCREENING TEST**

### **MOCK TEST - 8 PAPER - 1 DATED ON 20th JUNE 2026**

**Q6.** A 26-year-old professional tennis player presents with progressive difficulty serving overhead after repetitive shoulder strain. He reports deep posterior shoulder pain and weakness while attempting to externally rotate and elevate the arm. Physical examination reveals preserved initiation of abduction but marked weakness when maintaining the arm in abduction against resistance. External rotation is significantly impaired when the elbow is flexed at 90°. Sensation over the lateral shoulder is intact. MRI demonstrates an isolated tear involving a muscle inserting onto the greater tubercle of the humerus and primarily responsible for external rotation of the shoulder joint. Which of the following muscles is most likely injured?

- a) Supraspinatus
- b) Infraspinatus
- c) Teres major
- d) Subscapularis

**Q7.** A 32-year-old construction worker sustains a deep penetrating injury to the posterior triangle of the neck after falling onto exposed metal. Several weeks later, he complains of progressive difficulty lifting objects overhead and notices asymmetry of his shoulders while looking in the mirror. He denies numbness in the upper limb.

On examination, the right shoulder is mildly depressed at rest. During resisted shoulder shrugging, weakness is noted on the affected side. When asked to abduct the arm above the horizontal plane, he compensates with trunk movement and is unable to smoothly rotate the scapula upward. Winging is minimal at rest but becomes more apparent during attempted overhead elevation. Sternocleidomastoid strength is mildly reduced. Which of the following nerves is most likely injured?

- a) Long thoracic nerve
- b) Dorsal scapular nerve
- c) Spinal accessory nerve
- d) Suprascapular nerve

**Q8.** A 58-year-old man with uncontrolled hypertension suddenly develops diplopia and weakness of the left side of the body. On examination, the right eye is deviated “down and out,” with complete ptosis and a dilated nonreactive pupil. Motor examination reveals spastic hemiparesis involving the left upper and lower limbs with hyperreflexia and extensor plantar response. Sensory examination is normal. MRI demonstrates a focal infarct involving the ventral midbrain. Which syndrome is most consistent with this presentation?

- a) Benedikt syndrome
- b) Weber syndrome
- c) Claude syndrome
- d) Wallenberg syndrome

**Q9.** A 47-year-old woman presents with progressive numbness and weakness of the hands over several years. She reports frequent burns on her fingers while cooking because she often cannot





## MIST- MEDICAL INSTITUTE FOR SCREENING TEST

### **MOCK TEST - 8 PAPER - 1 DATED ON 20th JUNE 2026**

perceive pain or temperature properly. More recently, she has developed wasting of the intrinsic hand muscles and weakness of grip bilaterally. Neurological examination demonstrates:

Bilateral loss of pain and temperature sensation over the shoulders and upper limbs in a “cape-like” distribution

Preservation of vibration and proprioception

Atrophy of hand muscles with lower motor neuron signs at the level of the lesion

Hyperreflexia in the lower limbs

MRI reveals a fluid-filled cavitory lesion within the cervical spinal cord.

Which syndrome best explains these findings?

- a) Brown-Séquard syndrome
- b) Syringomyelia
- c) Tabes dorsalis
- d) Subacute combined degeneration

Q10. A 36-year-old man sustains a posterior dislocation of the knee during a road traffic accident. Following reduction, he develops severe pain in the leg with pallor and reduced distal pulses. Several hours later, he is unable to dorsiflex the foot and reports numbness over the dorsum of the foot and lateral aspect of the leg. Passive extension of the toes worsens pain significantly. Angiography demonstrates vascular compromise involving a vessel tethered proximally at the adductor hiatus and distally beneath the tendinous arch of soleus. Which of the following structures is most likely injured?

- a) Anterior tibial artery
- b) Popliteal artery
- c) Posterior tibial artery
- d) Peroneal artery

Q11. A 64-year-old man with long-standing atrial fibrillation suddenly develops severe abdominal pain that appears disproportionately intense compared with minimal abdominal findings on examination. Several hours later, he develops bloody diarrhea and signs of metabolic acidosis. CT angiography reveals acute occlusion of a major arterial branch arising from the abdominal aorta just inferior to the celiac trunk.

At surgery, ischemic necrosis predominantly involves:

- distal duodenum,
- jejunum,
- ileum,
- cecum,





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ascending colon,

and proximal two-thirds of the transverse colon.

Which of the following arteries is most likely occluded?

- a) Inferior mesenteric artery
- b) Superior mesenteric artery
- c) Celiac trunk
- d) Internal iliac artery

Q12. A 52-year-old man develops weakness in climbing stairs and difficulty rising from a seated position after a vascular procedure performed near the groin. Examination shows reduced patellar reflex and sensory loss over the medial leg. Which structure is most likely affected?

- a) Femoral nerve — lies lateral to the femoral artery and supplies the anterior compartment of the thigh
- b) Obturator nerve — passes through the obturator canal and supplies thigh adductors
- c) Tibial nerve — passes through the popliteal fossa and supplies posterior compartment of the leg
- d) Common peroneal nerve — winds around the neck of the fibula and supplies dorsiflexors of the foot

Q13. A 46-year-old man presents with acute appendicitis. During examination, maximal tenderness is elicited at a point located along a line joining the umbilicus to the anterior superior iliac spine. The surgeon explains that this surface landmark corresponds to the most common location of the base of the appendix. Which of the following anatomical landmarks is being described?

- a) Murphy point — surface landmark associated with gallbladder inflammation beneath the right costal margin
- b) McBurney point — junction of lateral one-third and medial two-thirds of the line from ASIS to umbilicus
- c) Midinguinal point — midpoint between ASIS and pubic symphysis overlying femoral artery
- d) Transpyloric plane — horizontal plane passing through L1 vertebral level

Q14. A 58-year-old man is brought to the emergency department after sudden collapse with severe tearing chest pain radiating to the back. CT angiography reveals a thoracic aortic aneurysm requiring urgent surgical planning. During discussion, the surgeon identifies the vertebral level at which the trachea bifurcates and the arch of the aorta begins and ends. Which of the following anatomical landmarks corresponds most closely to this level?

- a) Transpyloric plane
- b) Sternal angle (Angle of Louis)
- c) Xiphisternal joint
- d) Supracristal plane



## **MIST- MEDICAL INSTITUTE FOR SCREENING TEST**

### **MOCK TEST - 8 PAPER - 1 DATED ON 20th JUNE 2026**

- Q15. Regarding the embryology of the pharyngeal arches, all of the following statements are TRUE EXCEPT:
- a) The first pharyngeal arch gives rise to Meckel cartilage derivatives, muscles of mastication, and structures innervated primarily by the mandibular division of the trigeminal nerve.
  - b) The second pharyngeal arch contributes to muscles of facial expression, Reichert cartilage derivatives, and structures innervated by the facial nerve.
  - c) The third pharyngeal pouch gives rise to the inferior parathyroid glands and thymus, both of which migrate caudally during development.
  - d) The fourth pharyngeal arch is primarily innervated by the hypoglossal nerve and gives rise to intrinsic muscles of the tongue.
- Q16. Regarding embryologic development of the neural tube and its derivatives, all of the following statements are TRUE EXCEPT:
- a) Closure of the cranial neuropore normally occurs before closure of the caudal neuropore, and failure of caudal neuropore closure can result in spina bifida.
  - b) Neural crest cells contribute to structures including dorsal root ganglia, melanocytes, adrenal medulla, and components of the cardiac outflow tract.
  - c) Myelomeningocele is commonly associated with Arnold–Chiari II malformation due to traction-related downward displacement of hindbrain structures.
  - d) The neural tube is derived primarily from paraxial mesoderm, while neural crest cells arise from intermediate mesoderm adjacent to the developing somites.
- Q17. Regarding embryologic derivatives of the aortic arch system, all of the following statements are TRUE EXCEPT:
- a) The third aortic arch contributes to formation of the common carotid arteries and proximal internal carotid arteries.
  - b) The left fourth aortic arch contributes to part of the arch of the aorta, whereas the right fourth arch contributes to the proximal right subclavian artery.
  - c) The sixth aortic arch contributes to the proximal pulmonary arteries, and the left distal sixth arch persists as the ductus arteriosus.
  - d) The fifth aortic arch persists bilaterally in normal development and forms the definitive pulmonary trunk and descending thoracic aorta.
- Q18. A patient with severe emphysema develops progressive CO<sub>2</sub> retention. Which statement is incorrect?
- a) Central chemoreceptor responsiveness may decrease chronically
  - b) Hypoxic drive becomes more important for ventilation
  - c) Renal bicarbonate retention occurs as compensation
  - d) Chronic hypercapnia shifts blood pH toward severe alkalosis





## MIST- MEDICAL INSTITUTE FOR SCREENING TEST

### **MOCK TEST - 8 PAPER - 1 DATED ON 20th JUNE 2026**

**Q19.** A 58-year-old man with long-standing hypertension, poorly controlled diabetes mellitus, and a 40-pack-year smoking history is brought to the emergency department after experiencing sudden dizziness while climbing stairs carrying grocery bags. He reports generalized fatigue for several months but attributes it to poor sleep and work-related stress. His medications include metformin, amlodipine, and atorvastatin, although compliance is inconsistent.

On examination, he appears anxious and mildly diaphoretic. Blood pressure is 92/58 mm Hg, pulse is 118/min, and extremities are cool. Jugular venous pulse is difficult to visualize because of body habitus. Cardiac auscultation reveals a harsh systolic murmur radiating toward the neck. Lungs are clear. ECG shows left ventricular hypertrophy.

During exertion, he develops transient visual dimming and nearly collapses. The physician explains that despite increased sympathetic stimulation and tachycardia, his cardiovascular system is unable to appropriately augment systemic perfusion during increased metabolic demand.

Which physiological principle best explains his exertional symptoms?

- a) Increased venous return causing unrestricted increase in cardiac output
- b) Fixed limitation of stroke volume despite increased demand
- c) Reduced myocardial oxygen extraction during exercise
- d) Excess parasympathetic stimulation causing vasodilation

**Q20.** A 23-year-old medical student presents to the emergency department shortly before examinations complaining of tingling in the fingers, chest tightness, and lightheadedness. She reports sleeping poorly and consuming large amounts of coffee over the previous week. She denies fever, cough, or drug use.

On examination, respiratory rate is 32/min with deep respirations. Oxygen saturation is 100% on room air. ECG is normal. Arterial blood gas demonstrates elevated pH with reduced PaCO<sub>2</sub>. Neurologic examination reveals mild carpopedal spasm during blood pressure cuff inflation.

Despite normal oxygenation, cerebral symptoms persist until breathing normalizes.

Which physiological mechanism most directly explains her neurologic symptoms?

- a) Increased cerebral blood flow due to hypocapnia
- b) Cerebral vasoconstriction secondary to reduced carbon dioxide
- c) Increased oxygen unloading caused by respiratory alkalosis
- d) Reduced hemoglobin saturation from alveolar hypoventilation

**Q21.** A 67-year-old retired schoolteacher with chronic osteoarthritis presents with progressive lethargy and reduced appetite. He reports taking over-the-counter analgesics daily for years. Examination reveals dry mucous membranes and mild orthostatic hypotension. Laboratory studies show elevated serum creatinine and metabolic acidosis.

Urinalysis demonstrates impaired urinary acidification. Serum potassium is elevated. The nephrologist explains that despite severe acidemia, the kidneys are unable to adequately compensate.



## MIST- MEDICAL INSTITUTE FOR SCREENING TEST

### **MOCK TEST - 8 PAPER - 1 DATED ON 20th JUNE 2026**

Which renal process is most impaired in this patient?

- a) Renal bicarbonate filtration
- b) Hydrogen ion secretion and new bicarbonate generation
- c) Glucose filtration at the glomerulus
- d) Passive sodium diffusion in proximal tubule

Q22. A 31-year-old woman presents with progressive weight loss, palpitations, insomnia, and heat intolerance over several months. She recently changed jobs and attributes symptoms to stress. Examination reveals warm skin, fine tremor, and resting tachycardia.

Laboratory testing confirms elevated circulating thyroid hormone levels. During discussion with students, the endocrinologist notes that many symptoms are mediated not by direct hormone action alone, but by altered responsiveness of target tissues to circulating catecholamines.

Which physiological change most directly contributes to this effect?

- a) Downregulation of  $\beta$ -adrenergic receptors
- b) Increased  $\beta$ -adrenergic receptor expression
- c) Suppression of sympathetic nervous system activity
- d) Reduced mitochondrial oxygen consumption

Q23. Regarding cerebral autoregulation, all are correct EXCEPT:

- a) Maintains stable flow over varying BP
- b) Severe hypotension impairs autoregulation
- c) Hypercapnia increases cerebral blood flow
- d) Hypocapnia increases cerebral blood flow

Q24. Regarding cochlea, all are correct EXCEPT:

- a) Base detects high-frequency sounds
- b) Apex detects low-frequency sounds
- c) Organ of Corti contains hair cells
- d) Endolymph has low potassium concentration

Q25. Regarding bilirubin metabolism, all are correct EXCEPT:

- a) Unconjugated bilirubin is albumin-bound
- b) Conjugation occurs in liver
- c) Conjugated bilirubin is water-soluble
- d) Unconjugated bilirubin appears in urine normally





## MIST- MEDICAL INSTITUTE FOR SCREENING TEST

### **MOCK TEST - 8 PAPER - 1 DATED ON 20th JUNE 2026**

Q26. A 29-year-old pregnant woman at 33 weeks' gestation is referred for reduced fetal movements. She has chronic hypertension and intermittent medication noncompliance. Ultrasound demonstrates fetal growth restriction and markedly reduced amniotic fluid volume.

Fetal kidneys appear structurally intact. Doppler studies reveal increased placental vascular resistance. The obstetrician explains that the reduced fluid volume primarily reflects altered fetal physiology rather than a structural urinary tract abnormality.

Which mechanism most likely explains the oligohydramnios?

- a) Increased fetal swallowing
- b) Reduced fetal renal perfusion
- c) Increased maternal fluid transfer into placenta
- d) Excess fetal lung fluid absorption

Q27. A 42-year-old man with progressive muscle weakness notices that symptoms worsen throughout the day, especially after prolonged activity. He has difficulty chewing late in meals and complains of intermittent diplopia in the evenings. Neurologic examination in the morning is nearly normal except for mild ptosis. Repetitive activity worsens weakness. Reflexes and sensation are preserved. Which physiological abnormality best explains this disorder?

- a) Failure of presynaptic dopamine release
- b) Autoimmune destruction of postsynaptic acetylcholine receptors
- c) Peripheral demyelination of sensory nerves
- d) Increased acetylcholine synthesis in motor neurons

Q28. A 68-year-old man with severe long-standing hypertension, concentric left ventricular hypertrophy, stage III chronic kidney disease, and poorly controlled diabetes presents with progressive exertional dyspnea and episodic presyncope. He reports that symptoms worsen particularly when climbing stairs after meals. Examination reveals delayed carotid upstroke, a harsh systolic murmur radiating to the carotids, bibasal crackles, and mild peripheral edema.

Echocardiography demonstrates severe calcific aortic stenosis with preserved ejection fraction. During exercise testing, despite marked sympathetic activation and tachycardia, cardiac output rises minimally. Shortly afterward, pulmonary capillary wedge pressure rises disproportionately.

Which physiological mechanism most directly explains this patient's exertional decompensation?

- a) Excessive reduction in systemic vascular resistance causing unrestricted ventricular ejection
- b) Fixed obstruction causing inability to augment stroke volume despite increased preload and sympathetic drive
- c) Reduced myocardial oxygen extraction during exercise causing reflex bradycardia
- d) Increased ventricular compliance allowing excessive diastolic filling without pressure elevation



## MIST- MEDICAL INSTITUTE FOR SCREENING TEST

### **MOCK TEST - 8 PAPER - 1 DATED ON 20th JUNE 2026**

Q29. A 19-year-old man with type 1 diabetes presents with vomiting, abdominal pain, severe dehydration, and deep labored breathing. Laboratory studies reveal:

Glucose: 540 mg/dL

pH: 7.08

HCO<sub>3</sub><sup>-</sup>: markedly reduced

Positive ketones

Despite respiratory compensation, acidemia persists.

Which physiological limitation prevents complete normalization of pH?

- a) Respiratory compensation cannot fully restore bicarbonate concentration
- b) Hyperventilation increases hydrogen ion generation
- c) Renal compensation occurs immediately but incompletely
- d) Ketone bodies directly inhibit pulmonary ventilation

Q30. A woman presents with nocturnal paresthesia in the lateral 3½ digits and thenar weakness. Tinel's and Phalen's tests are positive → diagnosis: carpal tunnel syndrome. Compression occurs under which structure?

- a) Flexor retinaculum
- b) Extensor retinaculum
- c) Deltoid ligament
- d) Annular ligament

Q31. A 63-year-old man with long-standing ischemic heart disease and progressive exertional dyspnea is admitted for worsening lower-limb edema and abdominal distension. He reports increasing fatigue over several months and difficulty sleeping flat at night. His medications include furosemide, carvedilol, spironolactone, and aspirin, although adherence is inconsistent.

On examination, pulse is 92/min and regular, blood pressure is 106/68 mm Hg, and mild hepatomegaly is present. Cardiac auscultation reveals a pansystolic murmur increasing during inspiration. Bibasal crackles are absent. Jugular venous pulsation is elevated.

During bedside examination, a prominent venous pulsation is observed occurring synchronously with ventricular systole and carotid pulsation. The waveform becomes more pronounced during inspiration. The physician explains that this abnormal venous waveform reflects transmission of right ventricular systolic pressure into the right atrium due to valvular incompetence rather than altered atrial contraction.

Which of the following jugular venous pulse abnormalities is most likely present?

- a) Giant a wave
- b) Cannon a wave
- c) Prominent v wave
- d) Absent y descent



## MIST- MEDICAL INSTITUTE FOR SCREENING TEST

### **MOCK TEST - 8 PAPER - 1 DATED ON 20th JUNE 2026**

Q32. A 34-year-old man is enrolled in a research study investigating skeletal muscle energetics after prolonged immobilization following a lower-limb fracture. During electrically induced contraction of isolated muscle fibers, investigators observe that force generation progressively declines despite preserved action potential propagation and normal calcium release from the sarcoplasmic reticulum.

Biochemical analysis demonstrates ATP depletion within the muscle fibers. Electron microscopy shows persistent attachment between actin and myosin filaments with failure of cross-bridge detachment. The investigators explain that one step in the cross-bridge cycle is critically dependent on ATP binding rather than ATP hydrolysis itself.

Which of the following events most directly requires ATP binding in the sarcomere?

- a) Exposure of myosin-binding sites on actin by displacement of tropomyosin
- b) Release of calcium from the sarcoplasmic reticulum through ryanodine receptors
- c) Detachment of myosin heads from actin following the power stroke
- d) Sliding of thin filaments caused by shortening of actin filaments themselves

Q33. A 49-year-old man presents with chronic epigastric discomfort, recurrent diarrhea, and multiple episodes of refractory peptic ulcer disease over the past 2 years. He has been treated repeatedly with proton pump inhibitors with only temporary relief. He reports unintentional weight loss and intermittent steatorrhea.

Upper endoscopy reveals multiple ulcers extending beyond the first part of the duodenum. Gastric pH remains extremely low despite prolonged fasting. Serum gastrin levels are markedly elevated. During a physiology discussion, the gastroenterologist explains that excessive acid production can paradoxically impair digestion and nutrient absorption despite preserved pancreatic enzyme synthesis.

Which of the following physiological mechanisms most directly explains the patient's steatorrhea?

- a) Excess gastric acid inactivates pancreatic lipase within the duodenum, impairing fat digestion
- b) Hypergastrinemia directly inhibits bile salt secretion from hepatocytes, preventing micelle formation
- c) Increased secretin release causes complete suppression of pancreatic bicarbonate secretion
- d) Chronic acid hypersecretion accelerates intestinal motility by stimulating enteric dopamine release

Q34. A 57-year-old accountant with long-standing obesity and type 2 diabetes mellitus presents for routine follow-up. Over the past several months, he has attempted multiple lifestyle modifications including intermittent fasting, increased dietary fiber intake, and evening walks after dinner. He reports that despite initially struggling with adherence, he recently noticed progressive reduction in meal size and gradual weight reduction after initiation of a new injectable medication prescribed by his endocrinologist.

He also mentions occasional early post-meal fullness and mild nausea, especially after large meals, but denies abdominal pain or diarrhea. His wife notes that he "doesn't snack constantly anymore." Past history





## MIST- MEDICAL INSTITUTE FOR SCREENING TEST

### **MOCK TEST - 8 PAPER - 1 DATED ON 20th JUNE 2026**

includes hypertension, dyslipidemia, fatty liver disease, and mild obstructive sleep apnea. Medications include atorvastatin, losartan, metformin, and the recently added injectable therapy.

Laboratory testing shows improved HbA1c compared with previous visits despite lower circulating postprandial insulin levels than expected for the degree of hyperglycemia seen before treatment. During discussion with residents, the endocrinologist notes that the medication acts through pathways normally activated after nutrient ingestion from the gastrointestinal tract and that its effects depend partly on ambient glucose concentration rather than constitutive receptor activation.

Which of the following physiological actions is most consistent with the endogenous hormone being mimicked by this therapy?

- a) Persistent stimulation of glucagon secretion irrespective of plasma glucose concentration
- b) Enhancement of insulin secretion in a glucose-dependent manner with additional effects on gastrointestinal motility and satiety pathways
- c) Direct activation of intracellular GLUT4 translocation independent of pancreatic hormone signaling
- d) Increased gastric hydrogen ion secretion mediated through enterochromaffin-like cell activation

Q35. A 7-year-old boy is brought to the emergency department because of progressive confusion, irritability, and episodic vomiting that began after a recent viral illness. His parents report that he had always disliked high-protein foods and would often become lethargic after large meals during family gatherings. Developmental milestones were mildly delayed, but previous evaluations were inconclusive.

On examination, he is disoriented and intermittently agitated. There is no jaundice or hepatosplenomegaly. Respiratory rate is elevated. Laboratory studies reveal:

- Markedly elevated plasma ammonia
- Low blood urea nitrogen
- Respiratory alkalosis
- Elevated glutamine levels

Urinary analysis demonstrates markedly increased orotic acid excretion. Liver transaminases are normal.

A metabolic specialist explains that the disorder results from failure to incorporate nitrogen into the urea cycle despite preserved mitochondrial carbamoyl phosphate production. Which of the following enzymes is most likely deficient?

- a) Carbamoyl phosphate synthetase I
- b) Ornithine transcarbamylase
- c) Argininosuccinate synthetase
- d) Arginase





## MIST- MEDICAL INSTITUTE FOR SCREENING TEST

### **MOCK TEST - 8 PAPER - 1 DATED ON 20th JUNE 2026**

- Q36. The photomicrograph shows a transverse section of kidney cortex containing a renal corpuscle and surrounding tubules. The labeled structure “X” is the clear space surrounding the glomerular tuft. Identify the labeled structure “X” and select the most appropriate option regarding it.
- Bowman’s space; collects the filtrate formed by ultrafiltration and directs it into the proximal convoluted tubule.
  - Glomerular capillaries; site of active reabsorption of glucose, amino acids and water.
  - Efferent arteriole; carries blood from the glomerulus to the peritubular capillaries and vasa recta.
  - Parietal layer of Bowman’s capsule; formed by podocytes and has filtration slits between foot processes.
- Q37. A 12-year-old boy presents with exercise intolerance, muscle cramps, and fatigue since childhood. biopsy (PAS stain) shows excessive glycogen accumulation in skeletal muscle fibers (as shown in the image). During an ischemic forearm exercise test, lactate levels do not increase significantly after exercise. What is the most likely enzyme deficiency in this patient?
- Glucose-6-phosphatase deficiency causing impaired hepatic glucose release during fasting
  - Muscle glycogen phosphorylase deficiency causing defective glycogen breakdown during exercise
  - Branching enzyme deficiency causing formation of poorly branched glycogen with hepatosplenomegaly
  - Debranching enzyme deficiency causing accumulation of limit dextrin-like glycogen structures
- Q38. Regarding glycogen metabolism and its regulation, all of the following statements are correct EXCEPT:
- Glycogen phosphorylase catalyzes phosphorolytic cleavage of  $\alpha$ -1,4 glycosidic bonds during glycogenolysis.
  - Glycogen synthase is the rate-limiting enzyme of glycogenesis and is activated in the dephosphorylated state.
  - Branching enzyme creates  $\alpha$ -1,6 glycosidic linkages, increasing glycogen solubility and accessibility for rapid mobilization.
  - Glycogen phosphorylase directly hydrolyzes  $\alpha$ -1,6 branch points during glycogen breakdown without requiring accessory enzymes.
- Q39. Regarding these urea cycle disorders and their biochemical abnormalities, all of the following statements are correct EXCEPT:
- Carbamoyl phosphate synthetase I deficiency causes severe hyperammonemia with low orotic acid because mitochondrial carbamoyl phosphate production is impaired despite intact pyrimidine synthesis pathways.
  - Ornithine transcarbamylase deficiency may produce elevated urinary orotic acid because excess mitochondrial carbamoyl phosphate is diverted into cytosolic pyrimidine synthesis.
  - Argininosuccinate synthetase deficiency is associated with citrullinemia due to impaired conversion of citrulline to argininosuccinate within the cytosolic portion of the urea cycle.



## MIST- MEDICAL INSTITUTE FOR SCREENING TEST

### **MOCK TEST - 8 PAPER - 1 DATED ON 20th JUNE 2026**

- d) Arginase deficiency typically produces the most severe neonatal hyperammonemia of all urea cycle disorders because arginine directly blocks mitochondrial ammonia detoxification at the first committed step.

Q40. Regarding glycogen metabolism and its regulation, all of the following statements are incorrect EXCEPT:

- a) Glycogen phosphorylase directly hydrolyzes  $\alpha$ -1,6 glycosidic branch points during glycogen degradation.
- b) Glycogen synthase is activated primarily by phosphorylation mediated through glucagon-dependent protein kinase pathways.
- c) Branching enzyme deficiency characteristically produces structurally normal glycogen with isolated exercise intolerance and second-wind phenomenon.
- d) Glycogen phosphorylase catalyzes phosphorolytic cleavage of  $\alpha$ -1,4 glycosidic bonds to generate glucose-1-phosphate during glycogenolysis.

Q41. All of the following statements are TRUE EXCEPT:

- a) Excess phenylalanine competitively impairs transport of other large neutral amino acids into the brain.
- b) Reduced tyrosine availability contributes to impaired catecholamine and melanin synthesis.
- c) Some forms of hyperphenylalaninemia involve tetrahydrobiopterin deficiency affecting neurotransmitter synthesis pathways.
- d) Neurologic dysfunction in PKU primarily results from increased homogentisic acid deposition within cerebral white matter.

Q42. Regarding lipoprotein metabolism, all of the following statements are TRUE EXCEPT:

- a) Apo C-II activates lipoprotein lipase, facilitating triglyceride hydrolysis from circulating chylomicrons and VLDL particles.
- b) Apo B48 is required for intestinal assembly and secretion of chylomicrons.
- c) LDL particles primarily deliver cholesterol to peripheral tissues through receptor-mediated uptake involving Apo B100 recognition.
- d) HDL functions primarily to transport dietary triglycerides from intestine to peripheral adipose tissue after meals.

Q43. Regarding purine metabolism and associated disorders, all of the following statements are TRUE EXCEPT:

- a) Hypoxanthine-guanine phosphoribosyltransferase deficiency impairs purine salvage and increases uric acid production.
- b) Excess phosphoribosyl pyrophosphate availability contributes to increased de novo purine synthesis in Lesch-Nyhan syndrome.
- c) Xanthine oxidase catalyzes the conversion of hypoxanthine and xanthine into uric acid.





## MIST- MEDICAL INSTITUTE FOR SCREENING TEST

### **MOCK TEST - 8 PAPER - 1 DATED ON 20th JUNE 2026**

- d) Adenosine deaminase deficiency characteristically produces self-mutilation behavior and severe hyperuricemia due to excessive purine degradation.

Q44. Oxidative phosphorylation and electron transport chain physiology, all of the following statements are TRUE EXCEPT:

- a) Complex IV transfers electrons to molecular oxygen, forming water at the terminal step of the electron transport chain.
- b) Cyanide poisoning inhibits cytochrome oxidase activity, impairing oxidative phosphorylation despite adequate oxygen delivery.
- c) Proton gradients generated across the inner mitochondrial membrane drive ATP synthesis through ATP synthase activity.
- d) Inhibition of oxidative phosphorylation decreases anaerobic glycolysis because pyruvate can no longer be converted into lactate.

Q45. Regarding fatty acid synthesis and its regulation, all of the following statements are TRUE EXCEPT:

- a) Acetyl-CoA carboxylase catalyzes the rate-limiting step of fatty acid synthesis by forming malonyl-CoA.
- b) Citrate transported from mitochondria into cytosol provides substrate for fatty acid synthesis.
- c) Malonyl-CoA inhibits carnitine palmitoyltransferase I, reducing mitochondrial fatty acid oxidation.
- d) Glucagon strongly activates acetyl-CoA carboxylase through phosphorylation, thereby stimulating lipogenesis during fasting.

Q46. Pentose phosphate pathway and red blood cell metabolism, all of the following statements are TRUE EXCEPT:

- a) The pentose phosphate pathway is a major source of NADPH in erythrocytes.
- b) Reduced glutathione protects erythrocytes from oxidative membrane injury.
- c) Glucose-6-phosphate dehydrogenase catalyzes the rate-limiting step of the pathway.
- d) Mature erythrocytes compensate for oxidative stress by increasing mitochondrial ATP production and oxidative phosphorylation.

Q47. An infant develops vomiting, jaundice, hepatomegaly, and cataracts after starting milk feeds. Which enzyme deficiency is most likely?

- a) Galactokinase
- b) Galactose-1-phosphate uridylyltransferase
- c) Aldolase B
- d) Glucose-6-phosphatase



## MIST- MEDICAL INSTITUTE FOR SCREENING TEST

### **MOCK TEST - 8 PAPER - 1 DATED ON 20th JUNE 2026**

- Q48. A child develops recurrent vomiting, hypoglycemia, and seizures after consuming fruit juice. There is no hyperglycemic response after fructose administration. Which enzyme is deficient?
- Fructokinase
  - Aldolase B
  - Hexokinase
  - Galactokinase
- Q49. A patient with chronic alcoholism presents with confusion, ophthalmoplegia, and ataxia. Deficiency of which coenzyme is responsible?
- Pyridoxal phosphate
  - Thiamine pyrophosphate
  - Tetrahydrofolate
  - Biotin
- Q50. A 24-year-old man presents with progressively enlarging painless cervical lymphadenopathy associated with intermittent fever, drenching night sweats, and unintentional weight loss over the past 4 months. Physical examination reveals multiple firm, non-tender cervical lymph nodes without hepatosplenomegaly. Excisional lymph node biopsy demonstrates large atypical binucleate giant cells with abundant eosinophilic cytoplasm and bilobed nuclei containing prominent eosinophilic nucleoli, producing the characteristic “owl-eye” appearance. The surrounding background contains reactive lymphocytes, eosinophils, plasma cells, and histiocytes. What is the most likely diagnosis?
- Burkitt lymphoma
  - Hodgkin lymphoma
  - Chronic lymphocytic leukemia
  - Follicular lymphoma
- Q51. All of the following are incorrect EXCEPT:
- IPSP usually occurs due to sodium influx.
  - EPSP increases the chance of action potential generation.
  - Chemical synapses are faster than electrical synapses.
  - Neurotransmitter release is independent of calcium.
- Q52. All of the following are incorrect EXCEPT:
- Acetylcholine is released from the postsynaptic membrane.
  - Myasthenia gravis is due to antibodies against acetylcholine receptors.
  - Botulinum toxin increases acetylcholine release.
  - Curare blocks muscarinic receptors at the neuromuscular junction.
- Q53. A patient with diabetic ketoacidosis has serum potassium of 5.8 mEq/L before treatment. Total body potassium is actually depleted. All of the following are incorrect EXCEPT:



## MIST- MEDICAL INSTITUTE FOR SCREENING TEST

### **MOCK TEST - 8 PAPER - 1 DATED ON 20th JUNE 2026**

- a) Insulin deficiency shifts potassium into cells.
- b) Acidosis and insulin deficiency can shift potassium out of cells.
- c) Total body potassium is always increased in DKA.
- d) Insulin treatment increases serum potassium further.

Q54. A patient eats a fatty meal. Gastric emptying becomes slow. All of the following are incorrect EXCEPT:

- a) Fat in duodenum accelerates gastric emptying.
- b) CCK slows gastric emptying after fatty meal.
- c) Secretin strongly stimulates gastric acid secretion.
- d) Gastric emptying is fastest after fatty meals.

Q55. A 63-year-old man with chronic kidney disease misses dialysis due to heavy rain. His family says he has been drinking coconut water and eating bananas because someone told him they are good for weakness. He presents with generalized weakness and an irregular pulse. ECG shows tall peaked T waves. Serum potassium is 6.8 mEq/L. What happens to the resting membrane potential?

- a) Depolarization
- b) Hyperpolarization
- c) Stabilization
- d) Repolarization

Q56. A 60-year-old retired bank employee had a stroke while reading the newspaper after breakfast. Initially, he had right-sided weakness. After a few weeks, his son notices that the affected arm and leg have become stiff. Dressing him has become difficult. Examination shows increased tone, exaggerated deep tendon reflexes, ankle clonus, and extensor plantar response. Which finding is expected?

- a) Spasticity
- b) Areflexia
- c) Fasciculation
- d) Hypotonia

Q57. A 46-year-old woman presents with fatigue, tingling in her feet, and difficulty walking in the dark. She has a history of chronic gastritis and poor appetite. Blood tests show macrocytic anemia. The physician explains that vitamin B12 requires intrinsic factor from the stomach and is absorbed in a specific part of the small intestine. Damage to that part can cause deficiency. Where is B12 absorbed?

- a) Ileum
- b) Duodenum
- c) Jejunum
- d) Colon





## MIST- MEDICAL INSTITUTE FOR SCREENING TEST

### **MOCK TEST - 8 PAPER - 1 DATED ON 20th JUNE 2026**

- Q58. A 42-year-old man visits the hospital because his ring no longer fits his finger and his shoe size has increased over the past few years. His wife says his facial appearance has changed slowly, with a prominent jaw and enlarged nose. He also complains of sweating, headaches, and joint pain. Old photographs confirm gradual facial changes. MRI suggests a pituitary adenoma secreting growth hormone. The doctor explains that many effects of growth hormone are mediated through a liver-derived growth factor. Which mediator is responsible?
- Somatomedin
  - Thyroxine
  - Glucagon
  - Oxytocin
- Q59. A 9-year-old boy is brought because his face looks swollen in the morning. His mother says the swelling is most noticeable around the eyes after waking up. He has gained weight despite reduced appetite. Urine appears frothy. Examination shows pedal edema. Urine test shows heavy protein loss. The doctor explains that loss of plasma proteins reduces a force that normally holds fluid inside blood vessels. Which pressure is decreased?
- Oncotic
  - Systolic
  - Filtration
  - Osmotic
- Q60. A 20-year-old college student complains of reduced hearing in the right ear after a severe cold. He says sounds are muffled, but he does not have dizziness. He recently used ear buds aggressively after swimming. Examination shows wax and mild middle ear congestion. A tuning fork test is done. In Weber test, sound lateralizes to the affected ear. The doctor explains that this pattern occurs when sound conduction through the external or middle ear is impaired. Which deafness is likely?
- Conductive
  - Sensorineural
  - Cortical
  - Vestibular
- Q61. A 3-month-old infant is brought because his head appears larger than usual. His mother says the baby feeds slowly and cries excessively. On examination, the anterior fontanelle is bulging. Imaging suggests hydrocephalus due to impaired cerebrospinal fluid circulation. The pediatrician explains that CSF is normally produced by the choroid plexus and absorbed into venous blood through specialized structures. Where is CSF absorbed?
- Arachnoid
  - Choroid
  - Pineal
  - Pituitary



## MIST- MEDICAL INSTITUTE FOR SCREENING TEST

### **MOCK TEST - 8 PAPER - 1 DATED ON 20th JUNE 2026**

- Q62. A 3-year-old child has severe watery diarrhea, sunken eyes, lethargy, and drinks poorly. What is the best initial management?
- Oral zinc only
  - Oral rehydration solution at home
  - Intravenous fluid resuscitation
  - Antidiarrheal medication
- Q63. A patient presents with undulant fever, malaise, sweating, and arthralgia after consuming unpasteurized dairy products. Which organism is the most likely cause?
- Brucella melitensis*
  - Leptospira interrogans*
  - Rickettsia rickettsii*
  - Yersinia pestis*
- Q64. A 9-year-old boy develops facial puffiness and cola-colored urine two weeks after a sore throat. His mother says he had fever and throat pain but did not complete treatment. Urine examination shows red blood cell casts. The doctor explains that immune complexes are deposited in the glomeruli after infection. Which mechanism is involved?
- Immune
  - Toxic
  - Ischemic
  - Genetic
- Q65. A 45-year-old man who drinks alcohol daily presents with mild right upper abdominal discomfort. He works late nights, often skips meals, and drinks with friends after work. Ultrasound shows enlarged fatty liver. The doctor explains that early alcoholic liver injury commonly causes accumulation of triglycerides within hepatocytes. Which change occurs?
- Steatosis
  - Metaplasia
  - Dysplasia
  - Atrophy
- Q66. A 22-year-old woman has vaginal bleeding, severe nausea, and a uterus larger than expected for gestational age. Ultrasound shows a "snowstorm" pattern. She recently changed prenatal vitamins and has mild seasonal allergies. Which genetic pattern is most likely?
- 69,XXY triploid conceptus with maternal and paternal chromosomes
  - 46,XX androgenetic conceptus with only paternal genetic material
  - 45,X conceptus due to monosomy X
  - 47,XX,+21 conceptus



## MIST- MEDICAL INSTITUTE FOR SCREENING TEST

### **MOCK TEST - 8 PAPER - 1 DATED ON 20th JUNE 2026**

- Q67. A 68-year-old man has progressive exertional dyspnea and dry cough. He has digital clubbing. He previously worked as an accountant, collects stamps, and recently started eating less salt. High-resolution CT shows basal subpleural reticulation. What is the most likely histologic pattern?
- Nonspecific interstitial pneumonia
  - Usual interstitial pneumonia
  - Organizing pneumonia
  - Hypersensitivity pneumonitis
- Q68. A 52-year-old man has fatigue, recurrent infections, pancytopenia, and massive splenomegaly. Bone marrow aspiration results in a dry tap. He recently started woodworking and eats a vegetarian diet. Bone marrow biopsy shows widely spaced lymphoid cells with abundant cytoplasm, giving a “fried egg” appearance. Peripheral smear shows lymphocytes with cytoplasmic projections. Which molecular abnormality is most characteristic?
- BCR-ABL1 fusion
  - BRAF V600E mutation
  - MYD88 L265P mutation
  - CCND1-IGH translocation
- Q69. A 59-year-old man has hematuria and a renal mass. He has mild obesity, recently bought a rowing machine, and takes fish oil supplements. Imaging shows multifocal bilateral renal lesions. Renal tumor shows papillary architecture with foamy macrophages in fibrovascular cores. Cytogenetics reveals trisomy 7 and trisomy 17. Which diagnosis is most likely?
- Clear cell renal cell carcinoma
  - Papillary renal cell carcinoma
  - Chromophobe renal cell carcinoma
  - Oncocytoma
- Q70. A 61-year-old woman has epigastric discomfort and early satiety. She has a history of chronic gastritis. She also drinks peppermint tea, recently traveled by train, and has a family history of kidney stones. Gastric biopsy shows small atypical lymphoid cells infiltrating and destroying gastric glands, forming lymphoepithelial lesions. What is the most likely underlying driver?
- Epstein-Barr virus infection
  - Chronic Helicobacter pylori infection
  - Germline CDH1 mutation
  - Chronic gluten exposure
- Q71. A 41-year-old nonsmoker has recurrent pneumonia in the same lung segment. He also has mild eczema, recently visited a beach, and takes no medications. Endobronchial lung tumor shows





## MIST- MEDICAL INSTITUTE FOR SCREENING TEST

### **MOCK TEST - 8 PAPER - 1 DATED ON 20th JUNE 2026**

organoid nests of uniform cells with salt-and-pepper chromatin. There is no necrosis and fewer than 2 mitoses per 2 mm<sup>2</sup>. Which diagnosis is most likely?

- a) Small cell carcinoma
- b) Typical carcinoid tumor
- c) Lung adenocarcinoma
- d) Squamous cell carcinoma

Q72. A 69-year-old man has nephrotic-range proteinuria, fatigue, and back pain. Serum protein electrophoresis shows a monoclonal spike. He recently began gardening and takes calcium carbonate. Kidney biopsy shows Congo red-positive glomerular deposits with apple-green birefringence under polarized light. Mass spectrometry identifies lambda light chains. Which type of amyloid is present?

- a) AA amyloid
- b) AL amyloid
- c) ATTR amyloid
- d) Beta-2 microglobulin amyloid

Q73. A 67-year-old man presents with painless testicular enlargement. He has no history of cryptorchidism. He grows tomatoes, drinks black coffee, and had a colonoscopy last year. Testicular tumor shows three cell populations: small lymphocyte-like cells, intermediate cells, and large multinucleated cells. There is no intratubular germ cell neoplasia. Which diagnosis is most likely?

- a) Classic seminoma
- b) Spermatocytic tumor
- c) Embryonal carcinoma
- d) Yolk sac tumor

Q74. A 51-year-old man presents with chronic diarrhea, weight loss, migratory arthralgias, and intermittent confusion. He works as a landscaper, recently bought hiking boots, and drinks unfiltered water occasionally. Small bowel biopsy shows villi expanded by foamy macrophages containing PAS-positive, diastase-resistant material. What organism is responsible?

- a) Giardia lamblia
- b) Mycobacterium avium complex
- c) Tropheryma whipplei
- d) Campylobacter jejuni

Q75. A 29-year-old woman with photosensitivity, oral ulcers, and recurrent miscarriages develops a new murmur. She recently changed shampoo, takes iron supplements, and has a cat. Blood cultures are negative. Gross mitral valve shows small sterile vegetations on both sides of the valve leaflets. What is the most likely diagnosis?

- a) Rheumatic heart disease
- b) Infective endocarditis
- c) Libman-Sacks endocarditis



## MIST- MEDICAL INSTITUTE FOR SCREENING TEST

### **MOCK TEST - 8 PAPER - 1 DATED ON 20th JUNE 2026**

d) Nonbacterial thrombotic endocarditis

Q76. A 45-year-old man has abdominal pain after meals, hypertension, renal infarcts, and livedo reticularis. He has chronic hepatitis B infection. He also recently adopted a dog and eats spicy food daily. Medium-sized muscular artery shows segmental transmural necrotizing inflammation with fibrinoid necrosis. Glomeruli are spared. Which diagnosis best fits?

- a) Granulomatosis with polyangiitis
- b) Microscopic polyangiitis
- c) Polyarteritis nodosa
- d) Takayasu arteritis

Q77. A 36-year-old man has episodic headaches, palpitations, sweating, and hypertension. He recently began intermittent fasting, owns a motorcycle, and drinks decaf coffee. Urinary metanephrines are elevated. Adrenal medullary tumor shows nests of polygonal cells arranged in a Zellballen pattern with sustentacular cells at the periphery. What is the diagnosis?

- a) Adrenocortical adenoma
- b) Pheochromocytoma
- c) Neuroblastoma
- d) Metastatic renal cell carcinoma

Q78. A 54-year-old woman with a BRCA1 mutation undergoes risk-reducing salpingo-oophorectomy. She has mild reflux, recently started Pilates, and her sister has migraines. Fallopian tube fimbria shows markedly atypical secretory epithelial cells with strong diffuse p53 staining and increased Ki-67. Which carcinoma is this lesion most strongly associated with?

- a) Ovarian mucinous carcinoma
- b) Low-grade serous carcinoma
- c) High-grade serous carcinoma
- d) Adult granulosa cell tumor

Q79. A 4-year-old child presents with an abdominal mass. The parents mention the child recently changed daycare, eats strawberries daily, and has a cousin with asthma. Imaging shows a unilateral renal mass. Renal tumor biopsy shows nests and cords of pale cells separated by delicate arborizing "chicken-wire" vasculature. No triphasic blastemal, epithelial, and stromal pattern is seen. Which diagnosis is most likely?

- a) Wilms tumor
- b) Clear cell sarcoma of kidney
- c) Congenital mesoblastic nephroma
- d) Malignant rhabdoid tumor



## MIST- MEDICAL INSTITUTE FOR SCREENING TEST

### **MOCK TEST - 8 PAPER - 1 DATED ON 20th JUNE 2026**

- Q80. A 58-year-old man has slowly progressive scaly plaques on the trunk. He tried antifungal cream, changed bedsheets, and stopped eating gluten without improvement. Skin biopsy shows atypical lymphocytes with cerebriform nuclei infiltrating the epidermis and forming Pautrier microabscesses. What is the most likely diagnosis?
- a) Psoriasis
  - b) Mycosis fungoides
  - c) Allergic contact dermatitis
  - d) Langerhans cell histiocytosis
- Q81. A 42-year-old man develops new-onset seizures. MRI shows a calcified frontal lobe mass. He also reports drinking energy drinks, working night shifts, and having mild seasonal allergies. Brain biopsy shows uniform round cells with perinuclear halos, delicate branching “chicken-wire” capillaries, and calcifications. Which molecular alteration is most characteristic?
- a) BRAF fusion
  - b) IDH mutation with 1p/19q codeletion
  - c) MYC amplification
  - d) SMARCB1 deletion
- Q82. A 39-year-old man has a thyroid nodule, episodic diarrhea, and flushing. His father had surgery for “a neck tumor,” and the patient recently started taking vitamin D. Serum calcitonin is elevated. Thyroid biopsy shows nests of polygonal tumor cells separated by amyloid-rich stroma. Congo red stain is positive. Which diagnosis is most likely?
- a) Papillary thyroid carcinoma
  - b) Follicular thyroid carcinoma
  - c) Medullary thyroid carcinoma
  - d) Anaplastic thyroid carcinoma
- Q83. A 47-year-old woman presents with a rapidly enlarging breast mass. She recently started yoga, has a pet parrot, and drinks soy milk daily. Mammography shows a large circumscribed mass. Breast biopsy shows leaf-like clefts lined by epithelium with markedly hypercellular stroma. What is the most likely diagnosis?
- a) Fibroadenoma
  - b) Phyllodes tumor
  - c) Intraductal papilloma
  - d) Mucinous carcinoma
- Q84. A 31-year-old woman is found to have a liver mass on imaging after a minor car accident. She uses oral contraceptives, drinks matcha daily, and has a family history of gallstones. Liver enzymes are normal. Gross liver specimen shows a well-circumscribed lesion with a central stellate scar. Microscopy shows nodular hepatocellular hyperplasia with abnormal thick-walled vessels and bile ductular reaction. Which diagnosis is most likely?





## MIST- MEDICAL INSTITUTE FOR SCREENING TEST

### **MOCK TEST - 8 PAPER - 1 DATED ON 20th JUNE 2026**

- a) Hepatocellular adenoma
- b) Focal nodular hyperplasia
- c) Fibrolamellar hepatocellular carcinoma
- d) Cavernous hemangioma

Q85. A 67-year-old woman has chronic watery nonbloody diarrhea. Colonoscopy is grossly normal. She takes ibuprofen for knee pain, a proton pump inhibitor for reflux, and recently bought a new brand of coffee. Stool cultures are negative. Colonic biopsy shows a thickened subepithelial collagen band beneath the surface epithelium with increased intraepithelial lymphocytes. What is the most likely diagnosis?

- a) Ulcerative colitis
- b) Crohn disease
- c) Collagenous colitis
- d) Celiac disease

Q86. A 48-year-old man from the Caribbean presents with lymphadenopathy, skin plaques, hypercalcemia, and fatigue. He also reports eating sushi twice weekly and recently changed laundry detergent. Flow cytometry shows CD4-positive mature T cells. Peripheral smear shows atypical lymphocytes with multilobated "flower-shaped" nuclei. Which infectious agent is most closely associated with this malignancy?

- a) EBV
- b) HHV-8
- c) HTLV-1
- d) HCV

Q87. A 34-year-old man has progressive dyspnea and nonproductive cough. He smokes one pack daily, owns three cats, drinks herbal tea, and recently started running marathons. CT shows upper-lobe cysts and nodules. Lung biopsy shows stellate bronchiolocentric nodules containing eosinophils and histiocytes with grooved nuclei. Immunostaining is positive for CD1a and langerin. What is the most likely diagnosis?

- a) Sarcoidosis
- b) Pulmonary Langerhans cell histiocytosis
- c) Hypersensitivity pneumonitis
- d) Respiratory bronchiolitis-associated interstitial lung disease

Q88. A 16-year-old boy presents with hematuria, mild periorbital edema, and hypertension. He recently returned from a school debate competition, eats a high-protein diet, and his uncle has gout. Serum C3 is persistently low. Electron microscopy of a renal biopsy shows extremely electron-dense, ribbon-like deposits within the glomerular basement membrane. Immunofluorescence shows strong C3 staining with little or no immunoglobulin. Which diagnosis best explains the biopsy?

- a) Poststreptococcal glomerulonephritis



## MIST- MEDICAL INSTITUTE FOR SCREENING TEST

### **MOCK TEST - 8 PAPER - 1 DATED ON 20th JUNE 2026**

- b) IgA nephropathy
- c) Dense deposit disease
- d) Membranous nephropathy

Q89. A 22-year-old woman with a history of allergic rhinitis attends a friend's birthday dinner. She eats shrimp, chocolate cake, and a new "herbal immunity booster." Ten minutes later, she develops generalized itching, facial swelling, wheezing, dizziness, and a sense of impending doom. Her friend says she recently changed laundry detergent, and the patient also mentions she forgot to drink enough water that day. Examination shows hypotension, tachycardia, urticaria, and bilateral wheeze. Which drug is the immediate drug of choice?

- a) Intramuscular epinephrine
- b) Intravenous hydrocortisone
- c) Oral diphenhydramine
- d) Nebulized salbutamol

Q90. A 48-year-old man with poorly controlled epilepsy is brought to the emergency department after a generalized tonic-clonic seizure lasting 9 minutes. His wife reports that he skipped breakfast, argued with his neighbor, and recently bought a new gaming console. He has frothing at the mouth, rhythmic limb jerking, urinary incontinence, and no return to baseline consciousness. Blood glucose is normal. Which drug is preferred for immediate seizure termination?

- a) Intravenous lorazepam
- b) Oral carbamazepine
- c) Intravenous phenytoin as the first medication
- d) Oral ethosuximide

Q91. A 9-year-old girl is brought because her teacher notices she "freezes" during class for several seconds. She drops her pencil, stares blankly, then resumes writing without confusion. Her mother says she eats slowly, dislikes carrots, and recently joined a dance class. EEG shows generalized 3-Hz spike-and-wave discharges. She has no tonic-clonic seizures. Which drug is the drug of choice?

- a) Ethosuximide
- b) Phenytoin
- c) Carbamazepine
- d) Gabapentin

Q92. A 61-year-old woman develops brief, electric-shock-like pain over the right cheek whenever she brushes her teeth, applies makeup, or drinks cold water. She recently changed toothpaste, has mild osteoarthritis, and watches cooking videos late at night. Neurologic examination is otherwise normal. MRI excludes a mass lesion. Which drug is the drug of choice?

- a) Carbamazepine
- b) Morphine
- c) Amitriptyline



## MIST- MEDICAL INSTITUTE FOR SCREENING TEST

### **MOCK TEST - 8 PAPER - 1 DATED ON 20th JUNE 2026**

d) Sumatriptan

Q93. A 35-year-old woman has fluctuating ptosis, diplopia, and difficulty chewing near the end of meals. She says symptoms worsen after long clinic shifts and improve after rest. She also reports drinking iced coffee, recently changing mascara, and sleeping with a fan on. Reflexes and sensation are normal. Antibodies against postsynaptic acetylcholine receptors are detected. Which drug is commonly used as first-line symptomatic therapy?

- a) Pyridostigmine
- b) Atropine
- c) Botulinum toxin
- d) Dantrolene

Q94. A 19-year-old man undergoing surgery for a sports-related shoulder injury suddenly develops rising end-tidal CO<sub>2</sub>, generalized muscle rigidity, tachycardia, hyperthermia, and dark urine after exposure to a volatile anesthetic. His uncle once had “a bad anesthesia reaction,” but the family also mentions the patient drank protein shakes and played soccer the day before surgery. Which drug should be given immediately?

- a) Dantrolene
- b) Succinylcholine
- c) Halothane
- d) Neostigmine

Q95. A 57-year-old woman on multiple medications develops palpitations and syncope after several days of vomiting. She recently started a macrolide antibiotic, drinks grapefruit juice, and has been dieting for a family wedding. ECG shows polymorphic ventricular tachycardia with twisting of QRS complexes around the baseline and prolonged QT interval. Which drug is the first-line pharmacologic treatment?

- a) Intravenous magnesium sulfate
- b) Intravenous adenosine
- c) Oral propranolol only
- d) Intravenous digoxin

Q96. A 44-year-old man presents with headache, abdominal pain, blurred vision, and confusion after drinking “homemade alcohol” at a roadside gathering. His friends say he also ate spicy food, argued about cricket, and slept poorly. Labs show high anion gap metabolic acidosis and increased osmolar gap. Fundoscopy is difficult due to poor cooperation. Which antidote is preferred?

- a) Fomepizole
- b) Naloxone
- c) Flumazenil
- d) Vitamin K





## MIST- MEDICAL INSTITUTE FOR SCREENING TEST

### **MOCK TEST - 8 PAPER - 1 DATED ON 20th JUNE 2026**

- Q97. 28-year-old man is found unresponsive in a parked car. He has pinpoint pupils, slow respirations, and cyanotic lips. His backpack contains protein bars, a phone charger, and a small packet of powder. His friends say he recently complained of back pain and had been using “strong tablets.” Oxygen is started. Which drug should be administered?
- a) Naloxone
  - b) Flumazenil
  - c) Atropine
  - d) Physostigmine
- Q98. A 46-year-old woman receives midazolam for a short endoscopic procedure. After the procedure she remains excessively sedated, but her oxygenation is adequate with support. She has no seizure disorder, no chronic benzodiazepine use, and no evidence of mixed overdose. Her husband mentions she recently bought a blue handbag and dislikes hospital blankets. Which drug can reverse the sedative effect?
- a) Flumazenil
  - b) Naloxone
  - c) Protamine sulfate
  - d) Pralidoxime
- Q99. Farmer is brought in with sweating, salivation, lacrimation, vomiting, diarrhea, bronchorrhea, bradycardia, pinpoint pupils, fasciculations, and weakness after spraying pesticide. His family says he had tea with extra sugar, forgot his lunch, and argued about crop prices. The odor of pesticide is present on his clothing. Which treatment combination is most appropriate?
- a) Atropine plus pralidoxime
  - b) Naloxone plus flumazenil
  - c) Vitamin K plus protamine
  - d) Deferoxamine plus calcium gluconate
- Q100. A 39-year-old man has episodic headaches, sweating, palpitations, and severe hypertension. He recently started intermittent fasting, owns a motorcycle, and drinks decaf coffee. Plasma metanephrines are elevated, and imaging shows an adrenal medullary mass. Surgery is planned. Which drug should be started before beta blockade?
- a) Phenoxybenzamine
  - b) Propranolol
  - c) Dopamine
  - d) Hydralazine alone
- Q101. A 7-year-old boy returns from a camping trip with fever, severe headache, myalgia, and a rash involving wrists, ankles, palms, and soles. His parents are worried because he is under 8 years old. He



## MIST- MEDICAL INSTITUTE FOR SCREENING TEST

### **MOCK TEST - 8 PAPER - 1 DATED ON 20th JUNE 2026**

also ate marshmallows near a river, lost his cap, and was bitten by several insects. The physician suspects a tickborne rickettsial disease. Which drug is the drug of choice?

- a) Doxycycline
- b) Amoxicillin
- c) Azithromycin
- d) Cefixime

Q102. A 24-year-old man has dysuria and scant clear urethral discharge. NAAT is positive for *Chlamydia trachomatis* and negative for gonorrhea. He says he recently changed gym shoes, drinks energy drinks, and has a mild cough from air conditioning. He is not allergic to tetracyclines. Which drug is the recommended first-line treatment?

- a) Doxycycline
- b) Ceftriaxone alone
- c) Benzathine penicillin G
- d) Metronidazole

Q103. A 27-year-old pregnant woman at 18 weeks has mucopurulent cervicitis. NAAT is positive for *Chlamydia trachomatis*. She recently started prenatal yoga, drinks mango juice daily, and complains that her neighbor's dog barks at night. She has no macrolide allergy. Which drug is preferred?

- a) Azithromycin
- b) Doxycycline
- c) Ciprofloxacin
- d) Gentamicin

Q104. 33-year-old man has a painless clean-based genital ulcer and painless inguinal lymphadenopathy. Dark-field microscopy is consistent with *Treponema pallidum*. He reports eating sushi twice weekly, has a tattoo, and recently changed his phone case. He has no penicillin allergy. Which drug is the drug of choice?

- a) Benzathine penicillin G
- b) Doxycycline as first choice despite no allergy
- c) Ceftriaxone plus azithromycin
- d) Metronidazole

Q105. A 41-year-old man with HIV presents with progressive dyspnea, dry cough, fever, and hypoxemia. Chest imaging shows diffuse bilateral interstitial infiltrates. He recently moved apartments, bought a new air purifier, and has been eating instant noodles because of work stress. CD4 count is  $82/\text{mm}^3$ . Which drug regimen is treatment of choice?

- a) Trimethoprim-sulfamethoxazole
- b) Amphotericin B
- c) Isoniazid
- d) Oseltamivir



## MIST- MEDICAL INSTITUTE FOR SCREENING TEST

### **MOCK TEST - 8 PAPER - 1 DATED ON 20th JUNE 2026**

- Q106. A 55-year-old man from a tropical region is found to have abdominal pain, intermittent diarrhea, wheezing, and eosinophilia. He is about to receive corticosteroids for presumed autoimmune disease. He also reports walking barefoot as a child, drinking herbal tea, and collecting coins. Stool exam suggests *Strongyloides stercoralis*. Which drug is first-line?
- a) Ivermectin
  - b) Praziquantel
  - c) Metronidazole
  - d) Chloroquine
- Q107. A 29-year-old man returns from a region with chloroquine-resistant malaria. He has fever, chills, headache, and thrombocytopenia. Thick smear shows *Plasmodium falciparum*. He is stable, able to take oral medication, and has no severe malaria features. He also complains that airplane food upset his stomach and that he lost his luggage. Which oral regimen is preferred if available?
- a) Artemether-lumefantrine
  - b) Chloroquine
  - c) Primaquine alone
  - d) Doxycycline alone
- Q108. A 31-year-old backpacker develops foul-smelling greasy diarrhea, bloating, flatulence, and weight loss after drinking untreated mountain stream water. He also ate roasted peanuts, slept in a tent with a torn zipper, and bought a wooden souvenir. Stool antigen is positive for *Giardia lamblia*. Which drug is an appropriate first-line option?
- a) Tinidazole
  - b) Praziquantel
  - c) Acyclovir
  - d) Ceftriaxone
- Q109. A 4-year-old child is brought after swallowing multiple red tablets from his grandmother's iron bottle. He has vomiting, abdominal pain, lethargy, metabolic acidosis, and very high serum iron. The family says he also ate biscuits, played with toy cars, and spilled juice on the carpet. Which antidote is indicated?
- a) Deferoxamine
  - b) Succimer
  - c) Naloxone
  - d) Flumazenil
- Q110. A 6-year-old child has intense perianal itching at night. His mother says he grinds his teeth, dislikes vegetables, and recently started kindergarten. The tape test shows oval eggs consistent with *Enterobius vermicularis*. Several family members have itching. Which medication is appropriate?





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### **MOCK TEST - 8 PAPER - 1 DATED ON 20th JUNE 2026**

- a) Albendazole
- b) Praziquantel
- c) Metronidazole
- d) Chloroquine

Q111. A dead body shows bluish-purple discoloration over dependent parts that does not blanch on pressure. What does this finding indicate?

- a) Early rigor mortis
- b) Fixed postmortem lividity
- c) Cadaveric spasm
- d) Marbling

Q112. A 40-year-old man develops diplopia, ptosis, dry mouth, dysphagia, descending weakness, and constipation after eating home-canned vegetables at a family picnic. He also drank lemonade, played cards, and complained about the weather. Sensation is intact, and he remains alert. Botulism is suspected. Which treatment should be given urgently?

- a) Heptavalent botulinum antitoxin
- b) Acyclovir
- c) Ceftriaxone alone
- d) Pyridostigmine

Q113. Regarding *Staphylococcus aureus*, all are true EXCEPT:

- a) It is a gram-positive coccus arranged in clusters, catalase positive, and commonly produces golden-yellow colonies on culture.
- b) It is coagulase positive and can cause skin abscesses, wound infections, pneumonia, osteomyelitis, septic arthritis, and toxic shock syndrome.
- c) Protein A is an important virulence factor because it binds the Fc portion of IgG and interferes with opsonization and phagocytosis.
- d) It is optochin sensitive and bile soluble, which are classical laboratory features used to identify pneumococci.

Q114. Regarding *Streptococcus pneumoniae*, all are true EXCEPT:

- a) It is an alpha-hemolytic, lancet-shaped, gram-positive diplococcus that commonly causes lobar pneumonia, meningitis, otitis media, and sinusitis.
- b) It has a polysaccharide capsule, which is a major virulence factor and is also the basis for pneumococcal vaccines.
- c) It is bile soluble and optochin sensitive, helping distinguish it from viridans streptococci.
- d) It is coagulase positive, forms golden-yellow colonies, and commonly appears as gram-positive cocci in clusters.



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### **MOCK TEST - 8 PAPER - 1 DATED ON 20th JUNE 2026**

Q115. Regarding cadaveric spasm, all are true EXCEPT:

- a) It is instantaneous stiffening of a group of muscles at the moment of death.
- b) It may involve muscles engaged in intense emotional or physical activity immediately before death.
- c) It may preserve the last act of life, such as grasping a weapon, grass, clothing, or other object.
- d) It develops gradually several hours after death, after primary flaccidity, and spreads in the same sequence as rigor mortis.

Q116. Regarding firearm entry wounds, all are true EXCEPT:

- a) Entry wounds commonly show inverted margins due to the bullet entering the body.
- b) An abrasion collar may be present around the entry wound because the bullet rubs against the skin edge.
- c) Close-range firing may produce blackening, tattooing, burning, and singeing depending on distance and weapon type.
- d) Exit wounds always have a clear abrasion collar, are always smaller than entry wounds, and always show inverted margins.

Q117. Regarding burns in forensic medicine, all are true EXCEPT:

- a) Pugilistic attitude may occur in burned bodies due to heat stiffening and contraction of muscles.
- b) Soot in the air passages suggests the person was breathing during the fire.
- c) Antemortem burns may show vital reaction such as redness, inflammation, and blister changes.
- d) Postmortem burns always show a true vital inflammatory line of redness because inflammation continues normally after death.

Q118. Regarding arsenic poisoning, all are true EXCEPT:

- a) Acute poisoning may cause severe abdominal pain, vomiting, diarrhea, dehydration, shock, and collapse.
- b) Chronic poisoning may cause skin pigmentation, hyperkeratosis, peripheral neuropathy, and Mees lines on nails.
- c) A garlic-like odor may be described in some cases of arsenic poisoning.
- d) Atropine is the specific chelating antidote because it directly binds arsenic and increases urinary excretion.

Q119. Regarding human identification, all are true EXCEPT:

- a) DNA profiling can be highly useful for identifying decomposed, mutilated, charred, or skeletonized bodies.
- b) Fingerprints are valuable for identification when friction ridge patterns are preserved.
- c) Dental records are especially useful in burnt, decomposed, or mutilated bodies because teeth resist destruction.



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### **MOCK TEST - 8 PAPER - 1 DATED ON 20th JUNE 2026**

- d) ABO blood group alone conclusively proves individual identity because only one person can have a particular blood group.

Q120. Regarding medicolegal autopsy, all are true EXCEPT:

- a) External examination, clothing examination, injury documentation, and internal examination are all important.
- b) Injuries should be described with site, size, shape, margins, direction, associated findings, and possible weapon correlation.
- c) Samples should be properly preserved, sealed, labeled, and documented to maintain chain of custody.
- d) The autopsy surgeon should ignore clothing, external injuries, trace evidence, and scene information, and examine only internal organs.

Q121. Drug of choice for treatment of worm infestation related with given activity:



- a) Tinidazole
  - b) Albendazole
  - c) Cotrimoxazole
  - d) Mebendazole
- Q122. A study done in population where H/O polio vaccination was asked from mothers of babies with polio and without polio. Identify study:
- a) Cohort study
  - b) Case control study
  - c) Observation study
  - d) Cross sectional study
- Q123. 45year male presented with H/O altered bowel habit, his stool examination confirms presence of occult blood. There is F/H/O colon cancer present, what is next step?
- a) Cystoscopy
  - b) Colposcopy
  - c) Colonoscopy
  - d) Cholangioscopy





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Q124. Total fertility rate is:

- a) Total Girl children born alive
- b) Total Son born alive
- c) Woman reaching to age to reproduce
- d) Total children born alive

Q125. Which contraceptive is contra-indicated in a lady with deep vein thrombosis?

- a) IUCD
- b) Barrier
- c) OCP
- d) LNG-IUS

Q126. Reported live births for population of 50,000 is 750 for year 2025, calculate crude birth rate:

- a) 15/1000
- b) 1.5/1000
- c) 15%
- d) 1.5%

Q127. 35year woman C/O irregular bleeding with pain in lower abdomen for 15 days following use Copper IUCD. on examination tread visible in vagina with purulent foul smell discharge and pelvic examination shows tenderness. USG reveals normally placed device with features suggestive of PID, next best step for management:

- a) Remove device immediately followed by antibiotics
- b) Reassurance as bleeding is due to IUCD
- c) Combined with OCP to stop bleeding and add antibiotics
- d) Give trial course of 7day antibiotic and access after 7 days for its removal

Q128. A survey done in remote area of north east district where high incidence of malaria cases were reported and it was concluded that health department doing its duty properly as quick diagnosis and treatment of cases but no proper actions taken about vector control, most affected area of primary health care :

- a) Community participation
- b) Equitable distribution



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- c) Intersectoral coordination
- d) Appropriate technology

Q129. All are true about ICDS centres except:

- a) Under control of ministry of women and child development
- b) Main beneficiary- 0-6year children
- c) Anganwadi workers are trained by MPW-F
- d) Growth monitoring of children

Q130. All centres are included under given program except:



- a) Sub centre
- b) CHC
- c) PHC
- d) Health and wellness centre

Q131. Management of 4month pregnant lady with 11.8gm HB:

- a) 60mg elemental iron, 500mcg folic acid once a day prophylactic
- b) 60mg elemental iron, 500mcg folic acid twice a day therapeutic
- c) 100mg elemental iron, 500mcg folic acid once a day prophylactic
- d) 100mg elemental iron, 500mcg folic acid twice a day therapeutic

Q132. 20year girl reported to clinic with anaemia and you have started her on iron therapy. All can be added to this girl for better iron absorption and faster HB rise except:

- a) Albendazole
- b) Green leafy vegetable
- c) Vitamin C
- d) Tea, coffee, wine





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Q133. Addition of khesari dal in yellow pulse to make money is example of:

- a) Food fortification
- b) Food adulteration
- c) Food intoxication
- d) Foo additive

Q134. Appropriate statistical test to compare average blood sugar level in individuals of 5 different metro city of India:

- a) Paired t test
- b) Unpaired t test
- c) ANOVA test
- d) Chi square test

Q135. Best diagram to represent changing prevalence of diabetes in India in last 10 years will be:

- a) Line chart
- b) Scattered diagram
- c) Histogram
- d) Bar chart

Q136. Mean BP of a specific town is measured and reported as 134mm hg with SD of 2mm and 68% population BP lie between 132- 136mm Hg, what does it signifies?

- a) 68% population is having BP within +/- of 2SD of mean
- b) 68% population is having BP within +/- of 1SD of mean
- c) 68% population is having BP within +/- of 3SD of mean
- d) 68% population is having BP below 134mmHg

Q137. Disposal of 200 ml empty Astymin SN infusion bottle will be in which colour bag?







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- a) Yellow
- b) Red
- c) Black
- d) Blue

Q138. After collapse of a building, 30 workers injured. Building pieces fell on a worker feet and injury is as given in image where worker is not able to stand and walk but vitals are stable, he will be included in which category of triage?



- a) Red
- b) Yellow
- c) Green
- d) Black

Q139. All are correct about SDG goals except:

- a) Duration – 2016-2030
- b) Given by WHO
- c) Global MMR- < 70/1lac by 2030
- d) NNMR- 12/1000LB

Q140. Arrange phases of family cycle in correct order:

- a) Dissolution- formation- extension- retraction
- b) Formation- dissolution-extension-retraction
- c) Formation-extension-retraction-dissolution
- d) Formation-extension-dissolution-retraction

Q141. Patient living in Delhi has been diagnosed as plasmodium vivex infection. Management protocol is:



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### **MOCK TEST - 8 PAPER - 1 DATED ON 20th JUNE 2026**

- a) Artemether Lumifentrine with single dose primaquine on D2
- b) Artesunate sulfadoxine pyrimethamine with single dose primaquine on D2
- c) Chloroquine with 14 days primaquine
- d) Chloroquine only

Q142. If Prevalence of hypertension in a community is 50%, correct statement will be:

- a) 12.5% are knowing their ds
- b) 6.25% are having treatment
- c) 6.25% are having adequate treatment
- d) 25% are having hypertension

Q143. 15year boy presenting with pleomorphic rash with centripetal distribution, incorrect about disease:

- a) Live vaccine can prevent this condition
- b) Vaccine strain- OKA
- c) Crust infective so period of isolation is till fall of all crust
- d) Can lead to congenital varicella syndrome of baby if maternal infection occurs in pregnancy

Q144. Incentive given under Nikshay Poshan Yojna is:

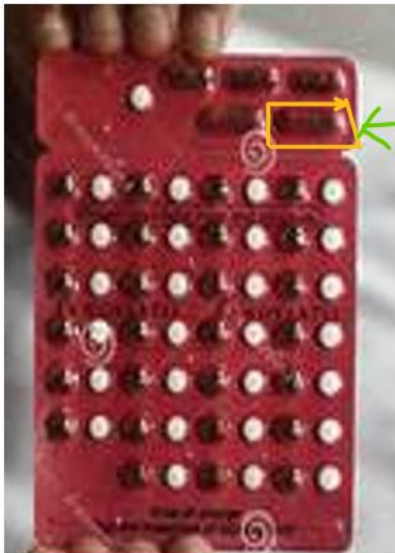
- a) 500RS/month till end of treatment
- b) 500RS/ week for first 2 month
- c) 1000RS/month till end of treatment
- d) 1000RS/ week for first 2 month

Q145. Incorrect about given program logo:



- a) Strategic pillar- detect, treat, prevent, build
- b) 4 S test for TB confirmation
- c) BPaLM- MDR treatment
- d) Designated microscopy centre- 1/1,00,000

Q146. Leprosy kit marked structure:



- a) Dapsone 100 mg
- b) Clofazimine 300 mg
- c) Rifampicin 300 mg
- d) Rifampicin 600 mg

Q147. All are correct about NHM except:



- a) ARSH clinic- care of sexual health in adolescent
- b) Urban ASHA POPULATION- 1000
- c) National rural health mission population coverage- < 50,000
- d) ASHA incentive under JSY in rural low performance states- 600/

Q148. Incorrect about given program:





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### **MOCK TEST - 8 PAPER - 1 DATED ON 20th JUNE 2026**



- a) Mother to child transmission without treatment – 30%
- b) Mother to child transmission with treatment – 10%
- c) ART drugs given to both mother during pregnancy and baby just after delivery
- d) 18<sup>th</sup> month ELISA report is final for baby

Q149. Correct about given logo:



- a) Surakshit matratva abhiyan – free ANC check up on 9<sup>th</sup> of every month at government sector
- b) Surakshit matratva abhiyan – free ANC check up on 9<sup>th</sup> of every month at private sector
- c) Single instalment of 5000/- after delivery of 2<sup>nd</sup> female child
- d) Single instalment of 6000/- after delivery of 2<sup>nd</sup> female child

Q150. Composition of marked tablet:



- a) Ethinyl estradiol- 30 mg, levonorgestrel- 150 mg
- b) Ethinyl estradiol- 30 mcg, levonorgestrel- 150 mcg
- c) Ethinyl estradiol- 150mg, levonorgestrel- 30 mg
- d) Ethinyl estradiol- 150 mcg, levonorgestrel- 30 mcg



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### **MOCK TEST - 8 PAPER - 1 DATED ON 20th JUNE 2026**

#### **ANSWERS**

|    |   |
|----|---|
| 1  | b |
| 2  | d |
| 3  | b |
| 4  | b |
| 5  | c |
| 6  | b |
| 7  | c |
| 8  | b |
| 9  | b |
| 10 | b |
| 11 | b |
| 12 | a |
| 13 | b |
| 14 | b |
| 15 | d |
| 16 | d |
| 17 | d |
| 18 | d |
| 19 | b |
| 20 | b |
| 21 | b |
| 22 | b |
| 23 | d |
| 24 | d |
| 25 | d |
| 26 | b |
| 27 | b |
| 28 | b |
| 29 | a |
| 30 | a |

|    |   |
|----|---|
| 31 | c |
| 32 | c |
| 33 | a |
| 34 | b |
| 35 | b |
| 36 | a |
| 37 | b |
| 38 | d |
| 39 | d |
| 40 | d |
| 41 | d |
| 42 | d |
| 43 | d |
| 44 | d |
| 45 | d |
| 46 | d |
| 47 | b |
| 48 | b |
| 49 | b |
| 50 | b |
| 51 | b |
| 52 | b |
| 53 | b |
| 54 | b |
| 55 | a |
| 56 | a |
| 57 | a |
| 58 | a |
| 59 | a |
| 60 | a |

|    |   |
|----|---|
| 61 | a |
| 62 | c |
| 63 | a |
| 64 | a |
| 65 | a |
| 66 | b |
| 67 | b |
| 68 | b |
| 69 | b |
| 70 | b |
| 71 | b |
| 72 | b |
| 73 | b |
| 74 | c |
| 75 | c |
| 76 | c |
| 77 | b |
| 78 | c |
| 79 | b |
| 80 | b |
| 81 | b |
| 82 | c |
| 83 | b |
| 84 | b |
| 85 | c |
| 86 | c |
| 87 | b |
| 88 | c |
| 89 | a |
| 90 | a |

|     |   |
|-----|---|
| 91  | a |
| 92  | a |
| 93  | a |
| 94  | a |
| 95  | a |
| 96  | a |
| 97  | a |
| 98  | a |
| 99  | a |
| 100 | a |
| 101 | a |
| 102 | a |
| 103 | a |
| 104 | a |
| 105 | a |
| 106 | a |
| 107 | a |
| 108 | a |
| 109 | a |
| 110 | a |
| 111 | b |
| 112 | a |
| 113 | d |
| 114 | d |
| 115 | d |
| 116 | d |
| 117 | d |
| 118 | d |
| 119 | d |
| 120 | d |

|     |   |
|-----|---|
| 121 | b |
| 122 | b |
| 123 | c |
| 124 | d |
| 125 | c |
| 126 | a |
| 127 | a |
| 128 | c |
| 129 | c |
| 130 | b |
| 131 | a |
| 132 | d |
| 133 | b |
| 134 | c |
| 135 | a |
| 136 | b |
| 137 | d |
| 138 | b |
| 139 | b |
| 140 | c |
| 141 | c |
| 142 | c |
| 143 | c |
| 144 | c |
| 145 | b |
| 146 | c |
| 147 | b |
| 148 | b |
| 149 | d |
| 150 | b |